Protocol for Treating Peyronie's Disease (IPP) Using Low-Intensity Shockwaves

MORENOVA

Device: MoreNova - C15 Circular Transducer
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1. Purpose and Scope

This protocol provides a standardized treatment plan for using low-intensity shockwaves to manage Peyronie's Disease (IPP). It is intended for healthcare providers using the MoreNova Device with the C15 Transducer to target the penile shaft, dorsal and lateral sides, and plaque
locations. This protocol applies to men aged 18-80 diagnosed with Peyronie's Disease.
For further information on clinical studies and efficacy, visit our Clinical Data Center.

Device Overview:
MoreNova is a portable Electromagnetic (EM) Low-Intensity Shockwave (LISW) device designed to reduce inflammation, dissolve calcifications, and stimulate angiogenesis, improving outcomes for various conditions in men and women.

2. Introduction

Peyronie's Disease is a condition characterized by fibrotic thickening or calcification of the tunica albuginea, often resulting in penile curvature, pain, and sexual dysfunction.
Traditional treatments, such as oral medications or surgery, are often invasive or have limited efficacy.
Low-Intensity Extracorporeal Shockwave Therapy (LiESWT) provides a non-invasive, effective alternative to alleviate plaque-related pain, improve penile curvature, and enhance sexual function by promoting neovascularization and plaque softening.

3. Scientific Review

Current Treatments:

- Oral therapy (e.g., tamoxifen, vitamin E): Limited efficacy, used off-label, and associated with side effects.

- Intralesional injections (e.g., collagenase, verapamil): Mixed results with significant side effects, such as hematomas and penile rupture.
- Surgery (e.g., plication, plaque excision): Effective for severe cases but invasive and with risks of erectile dysfunction.

Advantages of LiESWT:

- Non-invasive and painless.
- Promotes neovascularization and softens plaques via macrophage activation.
- Clinically proven to reduce plaque-related pain and improve curvature in some cases.
- No significant side effects with low-intensity applications.

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4. Patient Selection Criteria

Inclusion Criteria:

- Diagnosed with Peyronie's Disease for more than 3 months but less than 12 months.
- Penile curvature is between 30–90 degrees.
- Single identifiable plaque.
- Age 18–80 years.
- No history of penile surgery.

Exclusion Criteria:

- Penile curvature greater than 45 degrees.
- Previous surgery for Peyronie's Disease.
- Patients with blood coagulation disorders.
- Active lower urinary tract infections.
- The presence of a medical device sensitive to electromagnetic radiation (e.g., pacemaker).
- Past radiotherapy treatment of the treating region
- Known or suspected malignancy, at or near the treatment site or recovering from cancer during last year.

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5. Pre-Treatment Screening

To determine the patient's suitability for LiESWT, conduct the following:

- Comprehensive medical and sexual history review (trauma, plaque localization, cohabitation issues).

- Physical examination of the penile shaft for plaque palpation.
- Imaging: Use penile ultrasound to assess plaque size, location, and calcification.
- Questionnaires: PDQ (Peyronie's Disease Questionnaire) and IIEF to assess quality of life and psychological impact.

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6. Treatment Protocol

Preparation:

- Conduct a thorough medical history and physical examination.
- Counsel the patient about the procedure, benefits, risks, and expected outcomes.
- Obtain written informed consent.

Procedure:

- **Number of Sessions**: 6-10 sessions based on symptom severity.
- **Session Frequency**: Twice per week.
- **Session Duration**: 15 minutes.
- **Applications Per Session**: 4 (two angles targeting the middle and sides of the plaque).
- **Shocks Per Application**: 500.
- **Total Shocks Per Session**: 2,000.

Treatment Parameters:

- **Energy (HV) Settings**: 2.0 kV.
- **Frequency (FRQ)**: 2.0 Hz.

Post-Treatment Care:

- Advise patients to avoid sexual activity for 24 hours after each session.
- Recommend hydration and report any adverse events.

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7. Treatment Areas and Techniques

The treatment focuses on the following anatomical areas:

- Dorsal and lateral sides of the penile shaft.
- Plaque location (targeted at two angles).

Transducer Placement:

- Palpate and identify plaque location.

- Position the transducer at a 15 mm coverage diameter and apply gentle pressure.
- Administer 500 shocks per application area.

Adjustments by Severity:

1. **Mild Plaques (Volume <1 cm^3, curvature 30–45 degrees)**:
    - Apply standard protocol (2,000 shocks/session).
2. **Moderate Plaques (Volume 1–3 cm^3, curvature 45–70 degrees)** and **Severe Plaques (Volume >3 cm^3, curvature 70–90 degrees)**:
    - Consider applying 750 shocks per application (3,000 shocks/session).

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8. Patient Comfort and Safety

To ensure patient comfort and safety during the procedure:

- Position the patient supine with slight elevation of the pelvis for optimal access.
- Cover the transducer with a new transducer cover for hygiene.
- Use conductive gel to the transducer to minimize discomfort and enhance shockwave transmission.
- Monitor patient feedback during the session and adjust settings if necessary.

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9. Treatment Outcomes and Follow-Up

Expected Results:

- Pain reduction within 3-6 weeks.
- Potential softening of plaques and reduction in penile curvature.

Follow-Up Visits:

- Schedule bi-weekly follow-ups during the treatment period to monitor outcomes and address concerns.

Long-Term Maintenance:

- For sustained benefits, consider maintenance sessions every 6 months.

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10. Quality Control

Device

Calibration:

- Verify device settings before each session.
- The MoreNova Device recognizes preprogrammed parameters to ensure accuracy.

Clinician Training:

- Ensure all operators are trained in the use of the MoreNova Device and C15 Transducer.
- Clinicians should be familiar with this protocol and the "Instructions for Use" provided by the manufacturer.

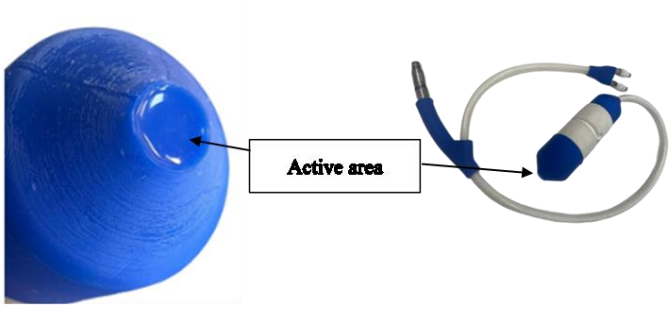
Checklist for Each Session:

- Confirm that the patient meets inclusion criteria and has no contraindications.
- Ensure the device and transducer are clean, sanitized, and functioning correctly.
- Verify that transducer placement aligns with specified anatomical areas.
- Fill with cold water and ensure during the treatment that the temperature stays in a proper range.

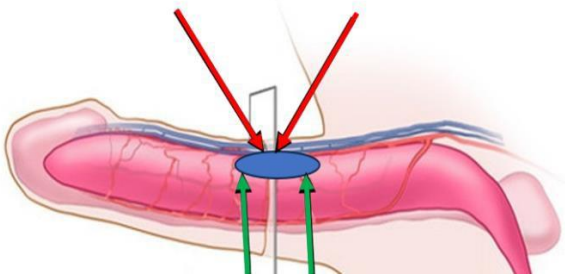
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11. Visuals and Diagrams

C15



- **Transducer placement technique and application angles.**



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12. References

For clinical studies and additional resources, visit the [Initia Clinical Data Center.]